How Systemic Racism and Structural Inequalities is main cause of Health Inequities for Racialized Populations in Canada

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Introduction

Canada is widely known for its universal public healthcare system which is often described as a source of national pride. The Canada Health Act of 1984 was built on the promise that every Canadian, regardless of who they are or where they live, should have access to medically necessary healthcare services without any financial barriers. However, despite this policy, not all Canadians experience the healthcare system in the same way. Racialized populations including Black, Indigenous, and other people of colour consistently experience worse health outcomes compared to white Canadians. These is not a coincidence, and they are the direct result of systemic racism and structural inequalities that are deeply rooted in Canadian society and its institutions.

Systemic racism refers to the ways in which racism operates through laws, policies, practices, and institutional norms, rather than just through individual attitudes and behaviours (Paradies et al., 2015). In the context of health, systemic racism shapes who gets access to care, how people are treated when they seek care, and the social conditions they live in that affect their health in the first place. These root factors including poverty, discrimination, housing insecurity, and barriers within the healthcare system itself work together to produce and maintain health inequities for racialized people in Canada.

This paper argues that systemic racism is the main cause of health inequities experienced by racialized populations in Canada, and that without immediate policy changes, these inequities will continue to get worse. The paper begins with a research question, followed by a review of existing academic literatures on this topic. It would also provides an analysis of policy solutions and concludes with urging all levels of government to take action.

Research Question

How do systemic racism and structural inequalities impact health outcomes for racialized populations in Canada, and what policy changes are most urgently needed to address these inequities?

Literature Review

To understand the relationship between systemic racism and health, I need to define a few key terms, In public health research, Race is a social construct developed to categorize people by physical traits such as skin color, and it has been used to justify racism, eugenics, experimentation, and other societal harms (University of Illinois Chicago School of Public Health, n.d.), Racialized populations refers to groups of people who are assigned a racial identity through social processes, and who face discrimination as a result of that identity. In Canada, this includes Black, Indigenous, South Asian, East Asian, Middle Eastern, Latin American, and other communities (Statistics Canada, 2022). Systemic racism, as defined by the (Public Health Agency of Canada, 2020), includes forms of racism that are embedded in systems, laws, policies, and practices that produce unfair treatment and oppression of racialized people, even when those systems appear to treat everyone equally on the surface.

The relationship between racism and poor health outcomes is well-documented in the academic literature. (Paradies et al., 2015) conducted a major meta-analysis of over 300 studies and found that racism both interpersonal and systemic racism is clearly associated with poorer mental and physical health outcomes. In Canada specifically, research has shown that racialized populations face higher rates of chronic diseases such as hypertension, diabetes, and depression (Galabuzi, 2004). These health inequities are not explained by genetics or individual behaviour. Instead, they are rooted in the social determinants of health, including factors like income, employment, housing, education, and access to healthcare, which are shaped by racism and discrimination.

The social determinants of health framework is important to understand how systemic racism produces poor health outcomes. It was introduced by the (World Health Organization, 2008) the framework recognizes that health is shaped not just by what happens inside hospitals, but by the conditions in which people are born, grow, live, work, and age. These conditions often put racialized people in Canada at a disadvantage. Research by (Galabuzi 2004) shows that racialized Canadians are more likely to live in poverty, work in unstable and low-paying jobs, and live in overcrowded or inadequate housing, all of which directly harm thier health. The COVID-19 pandemic made this issue impossible to ignore as racialized communities experienced higher rates of infection and death, not because of biological differences, but because they were more exposed and less protected. (Subedi et al., 2020).

Beyond social conditions, racialized individuals also experience racism directly within the healthcare system. Research shows that anti-Black racism and anti-Indigenous racism operate at multiple levels within healthcare system. At the interpersonal level, healthcare providers may hold conscious or unconscious biases that influence how they diagnose and treat patients. For example, Indigenous patients are sometimes stereotyped as intoxicated or non-compliant, which can lead to misdiagnosis or neglect (Allan & Smylie, 2015). The case of Brian Sinclair, a 45-year-old Indigenous man who died in a Winnipeg emergency room after waiting more than 30 hours without treatment for a treatable bladder infection, is a well-known and tragic example of how racial bias in healthcare can be deadly (Allan & Smylie, 2015).

At the organizational level, hospital and clinic policies can be indirectly discriminatory. Indirect discrimination occurs when policies that appear equal on the surface have unequal negative effects on people with certain protected characteristics, such as race (Ontario Human Rights Commission, 2013). For example, requiring certain forms of identification to access services, or failing to provide culturally safe care or language support services, can exclude racialized patients. Research by Nestel (2012) found that Canadian healthcare institutions have been slow to address anti-Black racism and often treat it as a personal or individual problem rather than a systemic one.

Another key issue is the lack of race-based data in Canada's healthcare system. Unlike countries such as the United Kingdom or the United States, Canada does not routinely collect data categorized by race in most healthcare settings. This means that the true scale of racial health inequities in Canada is not well captured in official statistics. A study found that only 23 peer-reviewed studies had been published on breast and cervical cancer among Black women in Canada which shows a massive gap in research that directly affects healthcare decision-making and policy development. **(Nnorom et al., 2019)** and without these data, it is very difficult to identify problems, target resources or measure the impact of policy interventions.

Indigenous peoples in Canada experience specific and a lot of health inequities which are shaped by centuries of colonialism, land dispossession, forced assimilation, and ongoing structural inequality. The residential school system which forcibly removed Indigenous children from their families and communities has had health impacts across generations, including higher rates of trauma, mental illness, substance use, and suicide (Truth and Reconciliation Commission of Canada, 2015). The Indian Act which was first passed in 1876 and still partially in effect today, created a federal system of control over Indigenous peoples identities, lands, and cultural practices which weakened thier social and cultural foundations of health. Research also shows that Indigenous peoples in Canada experience significantly lower life expectancy, higher rates of infant mortality, and greater burdens of chronic and infectious disease compared to non-Indigenous Canadians (Reading & Wien, 2009).

Some scholars have argued that the health inequities faced by racialized populations can be addressed through care that respects patients cultural backgrounds, that is, training healthcare providers to be more aware of and sensitive to the cultural backgrounds of their patients. While this is a good starting point, some argue that cultural competency alone is insufficient because it focuses on individual providers rather than structural change (Browne et al., 2012). Others point to the importance of increasing the diversity of the health workforce, arguing that representation matters and that when patients see providers who share their background and understand their experiences, health outcomes improve (Jetty et al., 2022). Some also emphasize the need for policy changes that address the root causes of health inequities, including anti-racism legislation, race-based data collection, and community-driven health programs (Nestel, 2012; Galabuzi, 2004).

In Summary, the literature makes a clear case that racialized health inequities in Canada are not accidental or avoidable. They are a result of specific systems and structures that can and must be changed through policy action.

Analysis

From a policy perspective, racial health inequities can be seen as a wicked problem because they are complex, interconnected, and difficult to solve through a single policy solution. The evidence reviewed in this essay leads to a clear conclusion that systemic racism is the primary cause of health inequities for racialized populations in Canada and addressing this problem requires urgent and structural policy change. While some may argue that health inequities are simply the result of individual lifestyle choices or socioeconomic differences that exist independently of race, this explanation does not fully explain the issue. According to (Galabuzi. 2004), racialized poverty and disadvantage in Canada are results of racial discrimination in labour markets, housing, education, and government policy. In other words, race and racism are not just correlated with poor health outcomes, they are linked to the structures of Canadian society. This analysis identifies three main policy interventions that are most urgently needed to address the health inequities experienced by racialized populations in Canada: mandatory race-based data collection, anti-racism training and accountability within the healthcare system, and increased investment in the social determinants of health for racialized communities.

The most important policy change needed is the mandatory collection of race-based health data across all provinces and territories. As I mentioned earlier, Canada lacks a standard race-based health data collection across the country, which makes it very difficult to identify where inequities exist, which populations are most affected, and whether the interventions are working. Several provinces, including Ontario, have started testing race-based data collection in healthcare, mainly due to the COVID-19 pandemic (Subedi et al., 2020). However, these efforts remain inconsistent and incomplete. A national policy that requires a standard race-based data collection inspired by the United Kingdom and the United States, would be the first step toward accountability and evidence-based action. The federal government, working through Health Canada and the Public Health Agency of Canada should lead this change while also collaborating with provincial health authorities and racialized communities.

Secondly, the healthcare system must implement mandatory anti-racism training for all healthcare providers and workers and combine with accountability for racist behaviour. A study shows that racial bias, both conscious and unconscious, affects clinical decisions and patient outcomes (Paradies et al., 2015; Allan & Smylie, 2015). Voluntary training programs are not enough because they depend on individual effort instead of the institution responsibility. A stronger approach would be to include anti-racism education in the training and licensing of all healthcare workers, from students to doctors and nurses. It is also important to have safe and anonymous ways for patients to report discrimination in healthcare settings with strong consequences for healthcare organizations that do not deal with racist behaviour. This is important to shift healthcare from viewing racism as an individual issue to viewing it as a systemic problem

Thirdly, addressing racialized health inequities requires investment in the social determinants of health for racialized communities. Even though training healthcare workers is important, it will not on its own close the health gap between racialized and non-racialized Canadians. As the literature shows, the main cause of health inequities are structural including income inequality, unstable jobs, poor housing, food insecurity, and under-resourced neighbourhoods. These are the conditions in which racialized people are forced to live, and they are the result of discriminatory policies in housing, immigration, and labour markets (Galabuzi, 2004). Addressing these root causes requires action across different policy areas, including affordable housing programs, income supports, and community health centres that are designed with racialized communities and reflect their specific needs and priorities.

These policy solutions are not separate, they work together and support each other. Collecting race-based data helps show where the biggest problems are and whether solutions are actually working. Anti-racism training in healthcare can make services safer and more accessible for racialized patients. Investments in the social determinants of health can help address the conditions that lead to poor health in the first place. Together, these three approaches provide a strong and evidence-based response to the urgent issue of racialized health inequities in Canada. Some people argue that policies for racialized populations are unnecessary or even cause division among Canadians, and suggest that universal social programs will reduce health inequities by helping all Canadians.

While universal programs are important, they are not enough on their own. Studies shows that race-based discrimination creates health disadvantages that cannot be explained only by income and social factors (Nazroo, 2003; Paradies et al., 2015). This means that even when income is similar, racialized people still experience worse health outcomes. This means that race-specific policy interventions are not just helpful, but they are important to achieve health equity in Canada.

Government suppourt these changes is growing but progress has been slow. The COVID-19 pandemic was as a major turning point that exposed how serious racial health inequities are and created a demand for action (Subedi et al., 2020). Advocacy from Black, Indigenous, and racialized communities and also research evidence has help increase awareness of the issue. The federal governments 2020 Anti-Racism Strategy and Ontario's collection of race-based COVID-19 data are signs of progress. The problem would requires consistent action from all levels of government

Conclusion

This paper has argued that systemic racism is the main cause of health inequities for racialized populations in Canada and that urgent policy change is needed to address this problem. Canada's publicly funded healthcare system was built on the principles of universality and accessibility, but for racialized Canadians, these goals have never been met. Research show that Black, Indigenous and other racialized people in Canada experience worse health outcomes, face barriers to accessing care and live in conditions shaped by racial discrimination, which lead to major health inequities.

Addressing these inequities is a matter of justice and a responsibility of the Canadian government. Three policy interventions are urgently needed: mandatory race-based health data collection, mandatory anti-racism training and accountability in healthcare, and investment in the social determinants of health for racialized communities. These solutions are based on evidence, in line with Canada's commitments to health equity and

necessary for building a healthcare system that works for everyone. There is a major gap between Canada's values and the lived experiences of racialized Canadians. Closing that gap will require deep and systemic change.

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